

A MANIFESTO FOR PEOPLE WHO SHIP

YOU AUTOMATED EVERYTHING EXCEPT YOURSELF.

You built pipelines so a thousand small, tested changes reach production safely. Then you went home and ran your own body on **no pipeline at all** — no commits, no tests, no observability, no rollback. This is an appeal to deploy the discipline you already trust to the one system you can never re-provision.

10–17%

lower risk of death, heart disease, cancer & diabetes in people who strength-train¹

30–60 min/wk

where the benefit curve bends — the highest-ROI hour you'll spend¹

2x /week

the global medical standard, a strong WHO recommendation²

 commit 00 — the premise

Your body is production. There is no staging copy.

Every other system you own is replaceable. Servers get re-provisioned, laptops get wiped and re-imaged, codebases get rewritten. Your skeleton and your muscle mass are the one deployment with **no rollback and no spare hardware**. And like any production system left unmonitored, it degrades silently. After 40, you lose bone at roughly 1% per year³ and muscle steadily alongside it — accruing, invisibly, exactly like the technical debt you keep promising to pay down. Nobody gets paged. The incident only fires years later, as a fall, a fracture, a frailty you can't refactor your way out of.

■ build — stop importing hype

Longevity fads are unaudited dependencies.

The wellness-industrial complex is selling you packages with impressive READMEs and no test coverage. NMN, resveratrol, metformin-for-the-healthy, exotic peptides, the \$40 mushroom coffee — the marketing implies production-grade, but pull up the actual evidence and you find **no supplement has been shown to extend human lifespan in a controlled trial.**⁴ Most of the splashy results are in yeast, worms, and mice; the human data is mixed, short, and underpowered. Reviewers keep landing on the same conclusion — **current human evidence doesn't support these compounds slowing aging**, and ordinary lifestyle measures have far stronger support.⁵

You would never `npm install` an unmaintained package from an anonymous author, skip the tests, and push it straight to prod. That is precisely the risk profile of a longevity stack: speculative, unregulated, and adopted on vibes. It's hype-driven development for your body.

Go where the evidence already shipped. Not where it might, someday, in a mouse.

■ deploy — where the ROI actually is

Strength training is the standard library.

While the supplements are still in preclinical, resistance training has decades of production use and the test coverage to match. A 2022 meta-analysis pooling sixteen cohort studies found that people who do muscle-strengthening activity carry a **10–17% lower risk of all-cause mortality, cardiovascular disease, total cancer, type 2 diabetes, and lung cancer.**¹ Not a surrogate marker in a petri dish — death, the actual endpoint, in humans.

And the dose is almost insultingly small. The same data shows the benefit curve bending hard at just **30 to 60 minutes a week.**¹ That's the ROI pitch in one line: roughly one Pomodoro, twice a week, against the leading causes of death. No engineer who optimizes for leverage should be able to walk past a return like that. It's why the World Health Organization, the American Heart Association, Mayo Clinic, and Harvard all converge on the same recommendation — train every major muscle group **at least twice a week.**^{2,6,7,8} This isn't a

biohacker's hot take. It is the settled, global, boring consensus of the medical establishment. The best dependencies usually are boring.

```
// hotfix — a critical patch for women
```

Bone density is a silent failure. Lifting is the fix.

This one deserves its own callout, because the people it helps most are the people most often told to stay light. Osteoporosis affects an estimated **eight million women in the US** — four times the number of men³ — and bone loss accelerates sharply at menopause. The stakes are not cosmetic: **six of ten people who break a hip never fully regain their independence.**³

For years the advice was to treat fragile bones gently. The **LIFTMOR trial** dismantled that. Postmenopausal women with low bone mass did twice-weekly, 30-minute sessions of genuinely **heavy lifting** — five sets of five at over 85% of their one-rep max — and over eight months **improved bone density in their spine and hips, improved function, and did it safely.**⁹ Bone, like any well-instrumented system, responds to load. Lift heavy, on purpose, and you write density back into the very places most likely to fracture: hip, spine, wrist.⁶

```
■ main — the pipeline, applied to you
```

You already know how to do this.

The whole reason this maps so cleanly is that strength is a CI/CD problem in disguise. **Small, frequent commits** beat one heroic effort — consistency over intensity. **Observability** means you log the sets and watch the trend, because what gets measured gets shipped. **Progressive overload** is just incrementing the version: a little more load, validated, every cycle. **Deloads are scheduled rollbacks.** And the meet, or simply the strong decades you get to keep, is the release. You don't need motivation. You need a pipeline — and building pipelines is the thing you're already good at.

01 Production over speculation. We invest where human outcomes are proven, not where mouse data is promising.

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- 02 **Ship small, ship often.** Two sessions a week, kept for years, beats a perfect program abandoned in a month.
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- 03 **Measure, don't guess.** Your body emits telemetry — load, reps, sleep, recovery. Read the dashboard.
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- 04 **Load is the feature, not the bug.** Bone and muscle adapt to stress. Heavy, progressive, well-coached load is the stimulus.
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- 05 **No rollback on a fracture.** We pay down physical debt now, while the patch is cheap.
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- 06 **Boring consensus beats shiny hype.** We trust the global medical standard over the supplement aisle.
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Deploy to yourself.

Past the longevity fads and the optimization theater, the highest-leverage health investment available to you is also the one with the deepest evidence base and the smallest time cost. Pick up something heavy, twice a week, for the rest of your life. Run the loop. Keep shipping.

```
$ cica --deploy self --strength --twice-weekly
```

REFERENCES

1. Momma H, et al. Muscle-strengthening activities are associated with lower risk and mortality in major non-communicable diseases. Br J Sports Med, 2022. pubmed.ncbi.nlm.nih.gov/35228201
2. WHO Guidelines on Physical Activity and Sedentary Behaviour, 2020 (strong recommendation: muscle-strengthening, all major muscle groups, 2+ days/week). ncbi.nlm.nih.gov/books/NBK566046

3. Harvard Health Publishing. Strength training builds more than muscles. [health.harvard.edu](https://www.health.harvard.edu)
4. Longevity Supplements: What Research Supports (review of NMN/resveratrol/rapamycin trial evidence). [summary of trial evidence](#)
5. The Conversation / Medical Xpress. Can NMN, NAD+ and resveratrol really slow aging? What the evidence says. theconversation.com
6. American Heart Association. Strength and Resistance Training Exercise. [heart.org](https://www.heart.org)
7. Mayo Clinic. Strength training: Get stronger, leaner, healthier. [mayoclinic.org](https://www.mayoclinic.org)
8. WHO 2020 guidelines, as above — global medical convergence on 2x/week.
9. Watson SL, et al. High-Intensity Resistance and Impact Training (LIFTMOR RCT). J Bone Miner Res, 2018;33(2):211-220. pubmed.ncbi.nlm.nih.gov/28975661

This manifesto is an evidence-informed argument, not medical advice. The mortality and disease figures come from observational cohort data (associations, not proof of causation); the bone-density findings are from a randomized trial. Before starting heavy training — especially with existing bone, joint, or cardiac conditions — talk to a clinician and, ideally, get coached on technique. Load is the medicine; dosed badly, it's also the injury.